

DIALOG Edmonton — Walk-In Card · July 10, 2026

One page. Everything verified 2026-07-01. Full reference: KOR-DIALOG-Edmonton-Structural-Outsourcing brief.

The goal (one sentence)

Leave as **the structural firm DIALOG Edmonton thinks of** for specialist/overflow and co-SE work — this is a relationship meeting; **there is no open seat to pitch.**

Who's across the table

Georg Josi — Partner, Structural Engineer, **Edmonton Studio Chair** (PhD, PEng). Healthcare background; Kaye Clinic is his favourite project. Possibly **Steven Oosterhof** — Partner (PhD/PE/SE), **voting member of the NBCC Standing Committee on Earthquake Design** (member — the chair is Perry Adebar, UBC). These are peers; talk engineer-to-engineer, never lecture code.

The one-line answer (if asked what we know about them)

DIALOG self-performs most structural — transit, bridges, mid-rise, civic, most healthcare — and brings in external structural where **a developer or design-lead architect procures it separately** (towers, Snøhetta-type projects), on **Alberta mega design-build hospitals as co-SE** (Kaye → Yolles/Jacobs *equal partners*; Calgary Cancer \$1.4B → Stantec), for **specialist innovation** (Fast+Epp mass timber, Equilibrium rammed earth), and **where a studio lacks the specialty** (Vancouver has no healthcare structural track record).

Five things to know

1. **Their AB structural bench is contracting** — Montgomery → Emeritus 2025, Sandra Renihan → transit PM — while the RPK merger (Aug 2025) added healthcare workload. Capacity pressure is real; probe it, don't assert it.
2. **The Kaye model is the door:** on the biggest AB design-build hospitals they've twice taken a co-equal external SE. The ask is to be *next on that list*.
3. **UHNBC (\$1.58B, Prince George)** is DIALOG BC + EllisDon in alliance — structural is *most likely their own BC scope* (no structural alliance participant was named; mechanical/electrical were). Ask about it as **capacity support**, not an open seat. Prince George is *low* seismicity — importance-factor angle, not seismic-intensity.
4. **Speak their language:** partner-owned B-Corp, alliance/IPD-fluent, "Community Wellbeing Framework," SE 2050. Say "integrated team," "all disciplines at the table" — **never "sub-consultant."** Bring the embodied-carbon/LCA story.
5. **The seismic wedge (exact wording):** *"Since May 1, 2024 Alberta designs to the NBC 2023 Alberta Edition — which for the first time brings Alberta post-disaster buildings under Article 4.1.8.23: essentially-elastic behaviour under a more frequent earthquake, 0.5% drift limit, $R_d \geq 2.0$. The 1.5 importance factor isn't new — this performance check is new to Alberta, and it's what coastal-BC practice has been built around for years."*

The four questions

1. "On **UHNBC** — is your BC team self-performing structural, or is there room for a BC healthcare-structural partner to help carry a \$1.6B tower?"
2. "When a hospital hits **Kaye / Calgary-Cancer scale** under design-build, how do you structure the co-SE — and who do you reach for?"
3. "Your Vancouver studio's structural strength is towers and transit — how are you covering **healthcare structural in BC**?"
4. "With the RPK merger and two partners stepping back, where's Edmonton's **structural bandwidth** tightest over the next 24 months?"

Do NOT say

1. X "You outsource hospital structural" — they self-perform at \$680M (Grande Prairie) and \$1B (Toronto Western). Only AB PCL design-builds went co-SE.
2. X Anything citing **Shangri-La as DIALOG-prime self-performing** — DIALOG was never the architect (James KM Cheng was); the structural is a Jones Kwong Kishi legacy credit (merged 2013). They know their own history.
3. X "The UHNBC structural seat is open" — most likely in-house at DIALOG BC. Ask; don't claim.
4. X "Oosterhof chairs the seismic committee" (member, not chair) · X "IE 1.5 is new" (since ~1990) · X "1% drift" (it's **0.5%**) · X "Alberta-unique rule" (national, new *to Alberta*).
5. X Vancouver studio has "only one structural engineer" (≥ 3 ; the true gap is **no healthcare track record**).
6. X Don't pitch against **Fast+Epp** (mass timber) or **RJC** (towers) — differentiate into healthcare/complex institutional instead.

Walking out, success looks like

Josi can say what KOR is *for*: **BC-seasoned post-disaster/healthcare structural depth + overflow capacity** — the Fast+Epp-on-The-Hive model, for their next co-SE or capacity pinch. Concrete next step secured: an intro to the BC studio (UHNBC), or “call us when the next Kaye-scale DB forms.”

Bring: APEGA Permit-to-Practice status · 2–3 nameable BC hospital/post-disaster references · the embodied-carbon/LCA story.